# Nivolumab · every 2 weeks · previously treated non-small-cell lung cancer (NSCLC)

Aim to control advanced lung cancer, relieve symptoms and prolong life where possible.

**Previously treated locally advanced or metastatic non-small-cell lung cancer (NSCLC).** · NCCP 00483 v13a · Document revised 6 November 2026

**Your personal plan:** this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

## My overall treatment

One treatment visit every 2 weeks; your team confirms duration.

**14** day view

Treatment timing in this window

**Treatment** — Day 1

**Nivolumab monotherapy**

Nivolumab 240 mg intravenous (IV) over 30 minutes OR 600 mg as a subcutaneous injection over 3–5 minutes on Day 1 every 2 weeks. Your team confirms one formulation; the whole visit includes checks and preparation. Continue until progression or unacceptable toxicity, with individual benefit and safety review.

## A typical 14-day cycle

| Day 1 | Day 2–14 |
|---|---|
| Your treatment visit | Between visits |

Timing may change after review. Side effects can occur on any day; there are no guaranteed safe days.

## My tests and checks

| When | What is checked | Timing / reason |
|---|---|---|
| **Before** | **Baseline safety**<br>NCCP | Blood count, kidney/liver, glucose and thyroid tests; clinical and medicine review. |
| **During** | **Bloods and symptoms**<br>NCCP | Blood count, kidney/liver and glucose before each cycle. Thyroid once a month. Cortisol if indicated. |
| **Response / after** | **Individual plan**<br>NCCP / individual plan | Review progression before cycle 8. Monitor immune effects for at least 5 months after the last dose. |

## Between treatments

- Report new symptoms promptly between visits and after treatment stops.
- Follow the written steroid or hormone-replacement plan. Do not restart a held medicine yourself.

Bring your medicine list, alert card, test dates and actual urgent contacts.

**When to get help**

**Call 112 / 999** for severe breathing difficulty, severe chest pain, collapse or a seizure.

**Call oncology immediately:** temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

**Oncology daytime:** 087 12345678 · **Out of hours:** 087 87654321

**EXAMPLE ONLY — team must replace both numbers before patient use.**



Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.

# What to watch for

You do not need to know which medicine caused a symptom before calling.

### 1 Hormones / nervous system

**Nivolumab**

Severe headache, confusion, weakness, dizziness or marked thirst need prompt advice.

### 3 Heart

**Nivolumab**

Chest pain, racing heartbeat or fainting needs urgent assessment.

### 5 Kidneys

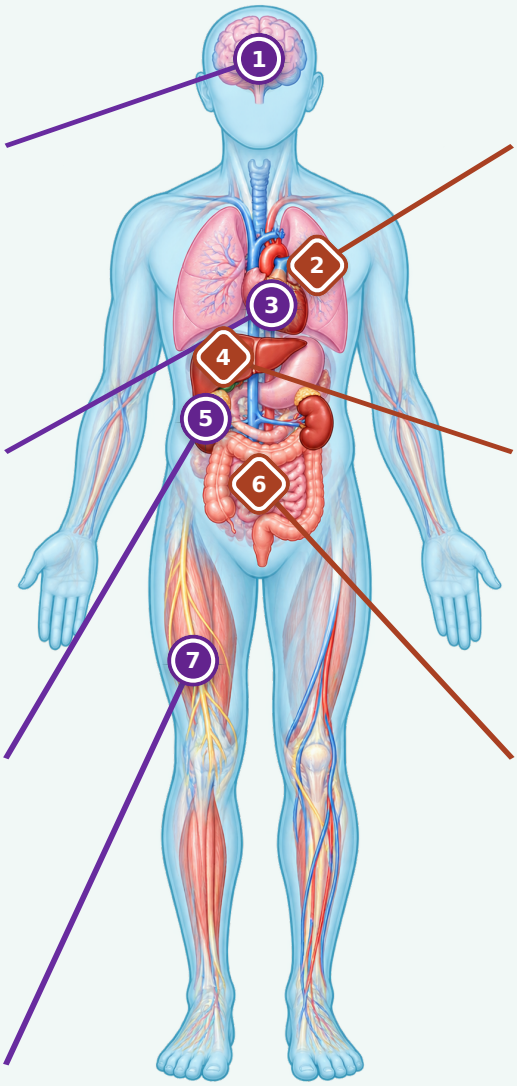
**Nivolumab**

Report much less urine, new swelling or a change in kidney tests.

### 7 Skin / muscles

**Nivolumab**

Spreading rash, blisters, severe weakness or muscle pain needs prompt review.




### 2 Lungs

**Nivolumab**

New cough or breathlessness can signal inflammation or another urgent cause.

### 4 Liver

**Nivolumab**

Yellow eyes/skin or dark urine may signal inflammation; tests also check the liver.

### 6 Bowel

**Nivolumab**

New or persistent diarrhoea, blood in stools or severe tummy pain needs urgent advice.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Contact the oncology team immediately for new or worsening breathlessness, chest pain, persistent diarrhoea, severe tummy pain, yellow skin/eyes, much less urine, severe headache, marked weakness, confusion, unusual thirst or passing much more urine, or a spreading/blistering rash. For severe breathing difficulty, collapse, seizure or other immediately dangerous symptoms, seek emergency care. Do not wait for the next infusion.

**When to get help**

**Call 112 / 999** for severe breathing difficulty, severe chest pain, collapse or a seizure.

**Call oncology immediately:** temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

**Oncology daytime:** 087 12345678 · **Out of hours:** 087 87654321

**EXAMPLE ONLY — team must replace both numbers before patient use.**



Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.

# Keep your treatment details to hand

**Previously treated locally advanced or metastatic non-small-cell lung cancer (NSCLC).** · NCCP 00483 v13a · Document revised 6 November 2026

Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

**FRONT / CONTACTS**

## My Treatment Card

SACTCheck

Name: _____

**Nivolumab · lung**

Every 2 weeks; confirm duration and current phase.

Phase: _____ Cycle plan: team to confirm

**Daytime (EXAMPLE)** 087 12345678

**Out of hours (EXAMPLE)** 087 87654321

**Unwell or fever? Call oncology now.**
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

**BACK / TREATMENT RECORD**

## My cycle record

Rolling record — follow your agreed treatment plan.

| Cycle | Date / initials |
|---|---|
| | |
| | |
| | |
| | |

My treatment information

Latest treatment date: _____

Plan changed? Ask the team to update this card.

CUT outer border

**FOLD centre — do not cut**

## Keep the record accurate

The team dates and initials each recorded treatment unit when complete. A cycle is complete after its final planned dose; a weekly-treatment stamp records that weekly dose. Delays, omitted doses or a changed plan need a note or a replacement card. A stamp does not approve the next treatment. Keep any medicine-specific alert card too.

## Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

### iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

### Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

## Your local contacts and resources

**Oncology daytime:** 087 12345678 · **Out of hours:** 087 87654321

**EXAMPLE ONLY — team must replace both numbers before patient use.**

Hospital / team: _____

Backup if the team cannot be reached: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.



Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.